



Go deeper than you could alone.
Empowering mental health practitioners.

Pre-revenue · 1 practice live India first 2 founders · 10 hrs a month

Most counsellors work blind. On the money they're owed. And on the clients quietly getting worse.

Deepen, outcome tracking for counsellors. It fixes both. Invoicing that runs itself, and a warning before a client slips away. Sold self-serve, grown by referral, run in ten founder-hours a month.

PAIN 1 · WHY THEY SIGN UP

"Month-end is spreadsheets, invoices made by hand, and awkward messages chasing payment. Some clients just disappear."

Your month closes in ten minutes.

Log sessions as you go. One click bills everyone. Rupee and dollar clients stay separate. Short sessions pro-rate themselves. Receipts, part-payments and credits are handled properly.

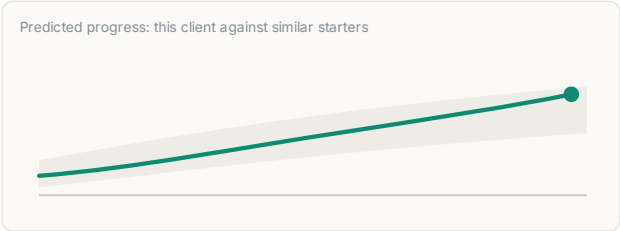
Admin is the loudest complaint in therapist burnout studies. It is not the deepest one.

PAIN 2 · WHY THEY STAY. NOBODY IN INDIA DOES THIS

"Some clients get worse. You don't see it until they stop replying, and then you guess for a very long time."

You'll know by session three.

A quick score each session, measured against where that client started. When someone slides, the note carries a flag and says why. It is information about the client, never a verdict on the counsellor.



Private by design. No employer, no platform, no supervisor ever sees it.

The market

India keeps no register of counsellors, so the number is reconstructed from the graduate pipeline and benchmarked against countries that do count. Base case shown; the model runs three scenarios and they differ by 3.4x at 2036.

54,263 Practising counsellors in India. Talk therapy as the primary paid job. Against 12,942 psychiatrists and 5,074 licensed clinical psychologists	24,418 In solo private practice. Everyone employed by a hospital, school, NGO or EAP leaves the universe—we never sell to organisations.	20,756 Digitally reachable. Self-serve, online, payable in rupees. The real top of funnel.	6,642 Able to afford Pro today. About a third. The rest are priced out, not unpersuaded.
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Workforce model, August 2026 · business/research/

The 98% is the customer. India graduates 14,759 psychology postgraduates a year against roughly 290 clinical psychology seats. Two per cent can ever be licensed; the rest go into unregulated counselling with no supervisor, no institutional quality signal and no way to know whether they are any good. **The regulatory gap that makes this market impossible to count is the same gap that creates the need.**

The affordability wall is the finding that matters. Most Indian therapists earn ₹3 lakh a year or less. Pro at ₹1,999 a month is ₹24,000 a year — **8% of gross revenue**. That is not a pricing objection to negotiate past. It is a structural ceiling that only moves when practitioner earnings move.

What moves the ceiling

The scenarios differ on whether a payer appears, not on training capacity. India already produces far more psychology postgraduates than practice absorbs.

CREDENTIAL DRIFT

65,664 counsellors · ₹23 cr

No counsellor pathway, parity unenforced. The credentialled pool grows faster than practice does.

STEADY FORMALISATION · BASE

121,827 counsellors · ₹54 cr

NCAHP registers counsellors, EAP demand compounds, school mandate partly enforced.

DEMAND UNLOCK

225,575 counsellors · ₹132 cr

A payer event fires — insurance parity enforced, a subsidised session scheme, or Tele-MANAS paying panel counsellors.

2036 figures at full penetration of the payable solo market. Every historical episode of fast workforce growth — UK IAPT, Australia's Better Access, the US post-parity decade — was triggered by a payer, not a training expansion. **The thing that grows this market is the same event that defines the third column.**

So India proves the product. It cannot make it big. Full penetration of the payable solo market is **₹24 crore today and ₹54 crore by 2036** on the base case. For scale, the combined FY24 revenue of India's ten best-known digital mental health platforms was about ₹103 crore. **100 paying users is 0.5% of the reachable base and entirely achievable. 10,000 would be a fifth of every solo counsellor in India we could reach, in 2036.** The path below is built on that.

Why the ceiling is not in India. Roughly 600,000 therapists in the US and 75,000 in the UK, in markets where the affordability wall does not exist. India is where the product earns its clinical credibility and its data. It is not where the revenue is.

Competition

Taken from public product pages, 31 July 2026. Re-check before quoting.

	deepen.	PractiPal India	PracFlow India	Zoho free India	Greenspace global	SimplePractice global · 200k users	TherapyNotes global
Invoicing and payments	✓	✓	✓	✓	·	✓	✓
Clinical session notes	✓	✓	✓	·	·	✓	✓
Tracks whether clients improve	✓	·	·	·	✓	✓	✓
Flags clients sliding back	✓	·	·	·	·	·	·
Predicts progress against similar clients	✓	·	·	·	·	·	·
Scheduling, reminders, client portal	·	✓	✓	✓	·	✓	✓
Price a month	₹999–1,999	₹1,499	₹1,199+	₹0	₹2,100	₹4.3–8.7k	₹6k

The honest read. Indian tools sell admin. Their "analytics" count revenue and cancellations, not client progress. The global players do track scores, but none flags deterioration or predicts trajectory, and at ₹4,000 to ₹9,000 a month they have priced themselves out of India. **Two rows are ours alone. The price column is the moat at home.**

Pricing

No free tier. 14-day trial. Annual by default. The first 25 users keep their price for three years.

THE WEDGE

Practice

₹999 a month · ₹9,990 a year

Billing and clinical records, working in fifteen minutes. Wins on setup, not features.

THE BUSINESS

Pro

₹1,999 a month · ₹19,990 a year

Everything above, plus progress tracking, flags and predicted progress. Priced against supervision and training, which counsellors already pay for.

Today a counsellor buys measurement (₹2,100) and billing (₹1,499) separately. That is ₹3,600 a month, and still no prediction. **Pro does both for ₹1,999.**

Revenue and costs

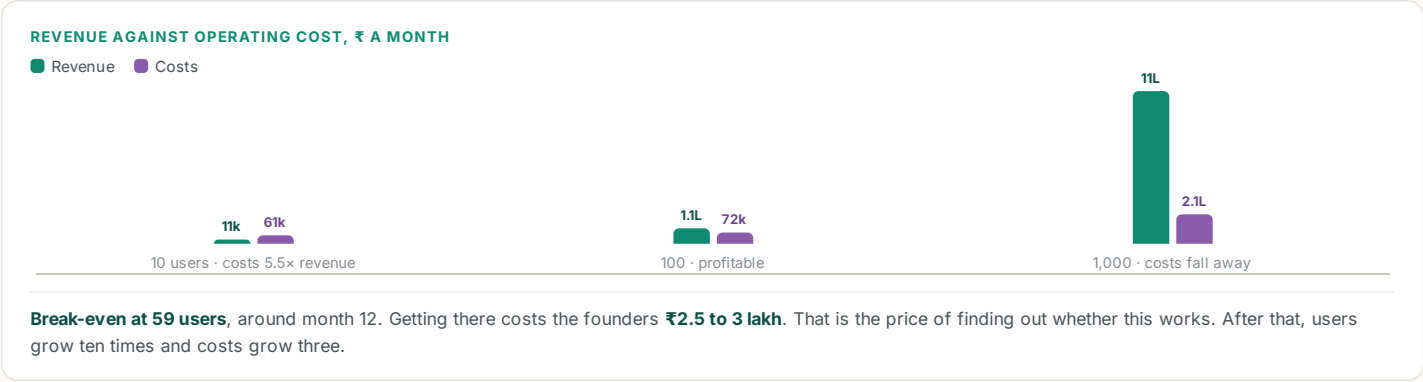
Three cases, each ten times the last. India economics at ₹1,100 blended per user, which assumes a 60/40 Practice/Pro mix. Worth knowing: the workforce model puts only about a third of the reachable base above the Pro affordability line, so 40% is the optimistic end. Targets, not forecasts.

MONTHLY COSTS. THREE LINES, NO OFFICE.

Only the tech role steps up: part-time contract to 100 users, full-time at 1,000.

₹ A MONTH	10 USERS LAUNCH	100 MONTH 18	1,000 YEAR 2–3
Servers and tools Neon, Vercel, Resend	6,000	12,000	40,000
Tech person 20 hrs a week, then full-time	40,000	40,000	1,20,000
Digital marketing content, webinar, email	15,000	20,000	50,000
Total cost	61,000	72,000	2,10,000
Revenue	11,000	1,10,000	11,00,000
Net a month	−50,000	+38,000	+8,90,000
Net margin	n/a	35%	81%

Founders take nothing until past 100 users. Marketing is digital only. Content and tooling first, small paid experiments out of the same line.



The path to 100,000 users

Four stages. The first two are India. The last two cannot be, and the workforce numbers are why. Grown through the founders' clinical network, a free monthly webinar, and content that compounds. No cold outreach, no demos, no selling to organisations.

INDIA · MONTHS 0–18 100 Prove the habit. 0.5% of the reachable base. Three-user beta, paid launch at month 4, break-even at 59. ₹1.1L a month. Still a side product.	INDIA · YEARS 2–3 1,000 Own the beachhead. About 5% of everyone in India we can reach and who can pay. Roughly ₹1.3Cr a year. This is close to India's practical ceiling, and it is the fork.	ENGLISH-SPEAKING · YEARS 3–5 10,000 Go abroad, because India realistically cannot supply this. Dollar pricing is already built, and the affordability wall does not exist in those markets. Needs HIPAA and GDPR, a team of five.	GLOBAL · YEARS 5–8 100,000 Own the category. The comparison pool improves with every user while incumbents run fixed curves. About \$12M a year at a blended \$10, plus a second line: licensing our own outcome measure to other vendors.
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The honest gate. Stages three and four abandon the side-product life, and they are not optional if the goal is size. That decision gets taken at the 1,000 mark with real numbers in hand, not before. The one thing that could change it is a payer event in India — enforced parity, a subsidised session scheme, or Tele-MANAS paying panel counsellors. That moves the ceiling from ₹54 crore to ₹132 crore and is entirely outside our control, so we plan without it and watch for it. North star throughout: counsellors who have recorded scores four weeks running.

Honesty line. No revenue today. One practice live, a co-founder's. Every projection is a target at a 60/40 Practice/Pro mix, not a forecast. Pricing stays unproven until the beta answers one question: what did you spend last year to become a better counsellor? Stages two to four depend on stage one working. Name decided: Deepen, domains registered, trademark search pending. The outcome measure is being brought in-house — the scales in the product today are third-party and are being replaced with our own, which is also what makes the licensing line in stage four possible.

Sources. India workforce counts are model output, not observation: reconstructed in business/research/india-counselling-workforce-2036.html from AISHE 2023–24 postgraduate out-turn (14,759, verified against the primary table), RCI register growth and WHO Atlas country profiles. Practitioner earnings, Zensible 2026 (n=285, metro-skewed). US counts, AutoNotes/BLS 2026. UK, BACP. SimplePractice user count is vendor-reported. Strategy detail sits in business/strategy/.